

LAKEVIEW IMAGING CENTER

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DIAGNOSTIC RADIOLOGY REPORT - FINAL

PATIENT:	Mary Lou Bevredeau	ACCESSION:	LIC-260120-4471
DOB:	06/21/1961	EXAM DATE:	January 20, 2026
SEX:	Female	ORDERING:	Anita Kessler, MD
INSURANCE:	Lakeshore Mutual Health	PRACTICE:	Northshore Spine Institute
MEMBER ID:	LMH-448219037	STATUS:	FINAL

EXAM:

MRI LUMBAR SPINE WITHOUT CONTRAST

INDICATION:

Low back pain with left leg radiating pain since June 2025. Persistent symptoms despite physical therapy. Evaluate for structural cause.

TECHNIQUE:

Multiplanar, multisequence MRI of the lumbar spine was performed on a 1.5 Tesla system without intravenous contrast. Sequences: sagittal T1, sagittal T2, sagittal STIR, axial T1 and axial T2 through L1 to S1.

COMPARISON:

None available.

FINDINGS:

Five lumbar-type vertebral bodies are assumed. Vertebral body heights are maintained. No marrow edema or suspicious marrow lesion. The conus medullaris terminates normally at the L1 level. Paraspinal soft tissues are unremarkable.

L1-L2: Unremarkable. No disc herniation, canal stenosis, or foraminal narrowing.

L2-L3: Unremarkable. No disc herniation, canal stenosis, or foraminal narrowing.

L3-L4: Unremarkable. No disc herniation, canal stenosis, or foraminal narrowing.

L4-5: Grade I anterolisthesis of L4 on L5, approximately 6 mm, in the setting of bilateral facet arthropathy with small facet effusions. Uncovering of the disc with broad-based bulge. Moderate left foraminal stenosis with contact of the exiting left L4 nerve root. Mild right foraminal narrowing. No significant central canal stenosis.

FINDINGS (CONTINUED):

L5-S1: Mild disc desiccation. No stenosis. No listhesis. No disc herniation. Facets are within normal limits for age.

Sacroiliac joints are partially imaged and grossly unremarkable. No retroperitoneal abnormality is identified on the included images.

IMPRESSION:

1. Grade I anterolisthesis of L4 on L5 at L4-5 with bilateral facet arthropathy and moderate left foraminal stenosis, with contact of the exiting left L4 nerve root. This correlates with the reported left leg symptoms.
2. Mild disc desiccation at L5-S1 without stenosis or listhesis.
3. L1-L2 through L3-L4 unremarkable.
4. Dynamic instability cannot be assessed on this static examination. Flexion-extension radiographs may be considered if clinically indicated.

Electronically signed by: Samuel Okafor, MD
Diagnostic Radiology, Lakeview Imaging Center
Date/Time Signed: January 20, 2026 4:42 PM CST